Session 7 (PROTECT & WRITE): Exercise sheet

Foundations of Clinical Research · Level 1 · Neudata · *#ClearDataClearImpact* · Week 7 · Saturday 12 December 2026 · online, 19:30–22:30

Name: _____ **Department:** _____

Type your answers into this Word file or print the PDF.

Part	Exercise	When	How
A	Integrity scenarios: green, amber or red?	20:25 (5 min)	Whole class, polls: 3 scenarios in class, 3 at home
B	Proposal builder 7: ethics, registration, feasibility, publication plan	20:30 (15 min)	The instructor models it on the running example; you write your own Part 4 at home
C	**Manuscript workshop 1**: the Results section for your FLUID-ICU question	21:35 (45 min)	30 min whole class on Q1, then 15 min on your own question. Finish at home

Files you need (sent by email): the results pack for your question (`Results_Pack_QN.pdf`), the manuscript template (`Manuscript_Template_IMRaD.docx`), Paper 1 (Boulet 2024) and Paper 2 (Hyun 2023).

Part A: Integrity scenarios: green, amber or red? (5 minutes, 20:25)

In class we vote on scenarios 1, 3 and 4 in a poll; scenarios 2, 5 and 6 are homework. For each scenario choose:

- **GREEN**: acceptable as described.
- **AMBER**: acceptable **only if** something is fixed or disclosed (say what).
- **RED**: research misconduct or a serious breach. Do not do it.

Write one sentence explaining your colour.

#	Scenario	Colour	Why?
1	Three patients' day-3 fluid charts are lost. A resident "remembers roughly" what their balances were and enters values so the dataset is complete.		
2	After seeing the results, the team removes two patients with "strange" fluid values because they weaken the association. The paper does not mention it.		
3	The head of department did not take part in the study but asks to be last author "because it is my department".		
4	(a) A nurse uses an AI chatbot to improve the English of the Discussion she wrote, checks every sentence, and states this in the Acknowledgements. (b) A colleague uploads the study spreadsheet (names and hospital numbers included) to a free public chatbot and asks it to "run the statistics".	(a)	
		(b)	
5	A cohort of patients with sepsis like FLUID-ICU is published as three papers (mortality, acute kidney injury and ICU stay) in three journals, none mentioning the others.		
6	Two paragraphs of a published review are copied word for word into your Introduction. The review is cited at the end of the paragraph, but there are no quotation marks.		

Part B: Proposal builder 7: ethics, registration, feasibility and publication plan

In class (20:30, 15 minutes): the instructor fills in this worksheet live on the shared screen for the running example (a retrospective cohort of fluid balance and 28-day mortality in adults with septic shock in our ICU). You suggest answers in the chat.

At home this week: fill it in for **your own** proposal. It becomes Part 4 of your written proposal and slides 5 (ethics and feasibility) and 6 (publication plan) of your presentation. Everyone submits the written proposal and the 6 slides by email before 19:30 on the day of Session 8 (`Proposal_Brief.pdf`).

B1. Ethics and consent

Question	Your answer
Which ethics committee will review the study? What type of review do you expect (exempt / expedited / full)? The committee decides.	
Who are the participants, and can they decide for themselves when they become eligible?	
Who gives consent (patient / legal representative / deferred consent / waiver requested)? Use the ICU consent flowchart (Session 7 slides).	
Who takes consent, when and where?	
What are the risks and burdens for participants, and how are they minimised?	
Does the study need registration (trials: yes, before the first patient, in ClinicalTrials.gov or another WHO registry; systematic reviews: PROSPERO; observational: recommended)?	

B2. Confidentiality

Question	Your answer
Which direct identifiers will you collect (if any), and why?	
How will each patient get a study code?	
Where is the key file (code to identity) kept, and who can open it?	
Where will the de-identified data be stored (institution-approved, password-protected)?	

B3. Feasibility check (tick and comment)

✓	Question	Comment
☐	**Time:** realistic timeline (ethics 1–3 months, recruitment, analysis, writing)?	
☐	**Money:** budget headings named (staff time, materials/tests, publication fee)?	
☐	**People:** who collects data (nights, weekends), who analyses, protected time?	
☐	**Patients:** how many eligible patients did your unit see last year? How many can you realistically enrol?	
☐	**Data:** are the key variables recorded routinely? How much is likely missing?	

Biggest feasibility risk: _____ One fix: _____

B4. Publication plan (slide 6 of your presentation)

Item	Your answer
Working title (from your PICO: design, I vs C, P, O; 10–16 words)	
Target journal (and a back-up), checked in DOAJ / NLM Catalog, with fees and licence known	
Reporting guideline (CONSORT / STROBE / STARD / PRISMA / SPIRIT / CARE)	
Registration needed? Where? (from B1)	
Authors (all four ICMJE criteria) and their roles (CRediT)	
Conflicts of interest and funding to declare	
Data-sharing statement (what, where, under which conditions)	
Conference where you would present an abstract first	

Structured-abstract skeleton. One line each. Results are the results you *expect to report* (format, not numbers).

Section	One line
Background	
Methods	
Results (expected)	e.g. "28-day mortality: n/N per group, RR (95% CI)"
Conclusion	Write the sentence you would use if the result is positive, and the one you would use if it is not.

Part C: Manuscript workshop 1: the Results section (45 minutes, 21:35)

You write the **Results section** for your FLUID-ICU question, using only your results pack. Methods, Introduction, Discussion and Abstract are optional (`Manuscript_Brief.pdf`).

Question	Topic	Results pack
Q1	Positive fluid balance by day 3 and 28-day mortality	`Results_Pack_Q1.pdf`
Q2	Cumulative fluid balance and acute kidney injury (AKI)	`Results_Pack_Q2.pdf`
Q3	Cumulative fluid balance and ventilator-free days	`Results_Pack_Q3.pdf`
Q4	Initial lactate and 28-day mortality, complete case vs multiple imputation	`Results_Pack_Q4.pdf`
Q5	Fluid balance and ICU length of stay among survivors	`Results_Pack_Q5.pdf`

How the 45 minutes are used

Clock	Who	Task
21:35–21:45	Whole class, Q1	Participants and the Table 1 sentence. You suggest sentences in the chat. The instructor types the best one
21:45–21:57	Whole class, Q1	Deaths per group, crude and adjusted OR with 95% CI
21:57–22:02	Whole class, Q1	Other analyses: Cox model and multiple imputation
22:02–22:05	Whole class	A volunteer checks every number against the pack
22:05–22:20	You	Start the Results for YOUR question, following the class draft

The class draft is sent as a PDF after the session. If you chose Q1, write it in your own words.

Results checklist (STROBE items)

- ☐ Participants first: numbers enrolled, analysed and excluded, with reasons (13)
- ☐ Table 1 in one or two sentences, with numbers. Missing data reported (14)
- ☐ Outcome numbers per group, with denominators (15)
- ☐ Main result: **crude and adjusted** estimate with 95% CI (16)
- ☐ Other analyses in your pack: sensitivity, imputation or subgroup (17)
- ☐ Table 1 and one main table or figure that stand alone
- ☐ No interpretation ("because", "suggests") and no causal words
- ☐ A working title, the simulated-data sentence and the AI-use statement

Question-specific reminders

- **Q3:** ventilator-free days are skewed and count death as zero.
- **Q4:** report both the complete-case and the multiple-imputation estimates.
- **Q5:** survivors only. Use medians and IQR.

Optional: Methods

If you also write the Methods, use STROBE items 4 to 12: design and setting, participants, variables, data sources, bias, study size ("all eligible patients in the cohort were included"), statistical methods and missing data. The facts are in your results pack, `SAP_FLUID-ICU.pdf` and `Data_Dictionary.pdf`.

Deliverable

Your Results section, named `Surname_Firstname_Results_QN.docx`, emailed to your reviewer and to the course lead (myluong1710@gmail.com) **by 23:59 on the day of Session 8**. Your reviewer is announced at the end of today's session.